

Customer Care Number
9599593622
9599593625

Accuracy Matters...89148733

Patient Name Mrs.SHUKLA KARMAKAR

Age/Sex 43 YRS/Female

Referred By DR. M DAS

Client Code/Name AP041308 LIFE CARE PATHOLOGY CENTRE

Ref. Lab/Hosp

Panel Address

Lab No

Reg Date

Sample Coll. Date

Sample Rec.Date

Report Date

26722501190003

19/Jan/2025 11:30AM

19/Jan/2025 10:28 AM

19/Jan/2025 05:28 PM

19/Jan/2025 07:19PM

12/2 MAHENDRA MUKHARJEE SARANU, UTTARPARA WEST BENGAL

.IMMUNO BIOCHEMISTRY-1

Test Name With Methodology	Result	Unit	Biological Ref.Interval
CCP Antibodies (Anti Cyclic Citrullinated Peptide Antibody)			
Anti CCP	332	U/ml	<17 Negative

Serum, Electro Chemi Luminescent Immuno Assay

COMMENT:

This test detects the presence of anti CCP antibodies in the serum. Anti-citrullinated protein/peptide antibodies (ACPAs) are autoantibodies (antibodies directed against one or more of the individual's own proteins) frequently detected in rheumatoid arthritis patients. Recently, these antibodies have turned up as powerful biomarkers, which are accepted as a major diagnostic tool in diagnosing rheumatoid arthritis (RA) already in a very early stage of disease.

In patients with symptoms of rheumatoid arthritis, testing positive for both CCP antibodies and RF suggest that a patient is likely to have rheumatoid arthritis. Testing positive for CCP antibodies and negative for RF may indicate that a patient is likely to develop rheumatoid arthritis in the future or is already in the early stages. Testing negative for both CCP antibodies and RF indicates that a patient is less likely to have rheumatoid arthritis.

Accuprobe
Diagnostics

Test Processed at:

Accuprobe Diagnostics, No. 2, Hemachandra
Naskar Road, Beliaghata. Kolkata 700010Dr. N Mondal, MD
(Consultant Biochemist)
Reg. No. WBMC-64023Dr. Arindam Das, MD, Path
(Consultant Pathologist)
Reg. No. WBMC-64631Dr Prashant Goyal
(Director &
Chief Pathologist)

Accuprobe Healthcare & Diagnostics Pvt. Ltd.

Corporate Office & Reference Lab:

B-14/7, Jhilmil Industrial Area, Delhi-110095

E: info@accuprobe.in | W: www.accuprobe.in

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Reg Date **19/Jan/2025 11:30AM**
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Report Date **19/Jan/2025 06:27PM**

Test Name With Methodology Result Unit Biological Ref.Interval

HAEMATOLOGY

Complete Blood Count (CBC)

Haemoglobin	12.6	gm/dl	12.0-15.0
Whole Blood EDTA, Cyanide free			
TLC (Total Leucocyte Count) /(WBC)	3.84	th/cumm	4.0-10.0
Whole Blood EDTA, Flow Cytometry			
DIFFERENTIAL LEUCOCYTE COUNT			
Polymorphs	59.1	%	40-80
Whole Blood EDTA Flowcytometry			
Lymphocytes	29.9	%	20-40
Flowcytometry			
Eosinophils	4.1	%	1-6
Flowcytometry			
Monocytes	6.8	%	2-10
Whole Blood EDTA Flowcytometry			
Basophils	0.1	%	0-1
Whole Blood EDTA Flowcytometry			
Absolute Neutrophil Count	2,269	/cumm	2000-7000
Whole Blood EDTA, Flowcytometry			
Absolute Lymphocyte Count.	1,148	/ μ L	1000.0 - 3000.0
Whole Blood EDTA, Flowcytometry			
Absolute Eosinophil Count	157	/cumm	20-500
Whole Blood EDTA, Flowcytometry			
Absolute Monocyte Count	261	/cumm	20-1000
Whole Blood EDTA, Flowcytometry			
Absolute Basophils Count	4	/cumm	20-100
Whole Blood EDTA, Flowcytometry			
RBC	4.31	millions/cmm	3.8-4.8
Whole Blood EDTA, Impedance			
HCT	41.5	%	36-46
Whole Blood EDTA, Calculated			
MCV	96.29	fl	83-101
Whole Blood EDTA, Calculated			
MCH	29.1	pg	27-32
Whole Blood EDTA, Calculated			
MCHC	30.4	g/dl	31.5-34.5
Whole Blood EDTA, Calculated			
Platelet Count	190	thou/ μ L	150-410
Whole Blood EDTA, Impedance			

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Referred By	DR. M DAS	Sample Rec.Date	19/Jan/2025 05:22 PM
Client Code/Name	AP041308 LIFE CARE PATHOLOGY CENTRE		
Ref. Lab/Hosp	Report Date 19/Jan/2025 06:27PM		
Panel Address	12/2 MAHENDRA MUKHARJEE SARANU , UTTARPARA WEST BENGAL		

MPV Calculated	16.2	fL	7.4-10.4
RDW- CV	14.6	%	11.6-14.0
Whole Blood EDTA, Flowcytometry			
RDW- SD	52.4	fL	35-56
Whole Blood EDTA, Flowcytometry			
PCT	0.24	%	0.10-0.28
Whole Blood EDTA, Flow Cytometry			
PDW	16.3	fL	9.0-17.0
Whole Blood EDTA, Calculated			
Neutrophil - Lymphocyte Ratio (NLR)	1.98	Ratio	
Calculated			
Lymphocyte - Monocyte Ratio (LMR)	4.40	Ratio	
Calculated			
Platelet - Lymphocyte Ratio (PLR)	165.48	Ratio	
Calculated			

Kindly correlate clinically. Advise for recheck from fresh sample in case, it is not correlation clinically, to rule out any pre-analytical error.

Referrance range according to Practical Haematology, Dacie & Lewis, 12th edition, 2012.

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Page 3 of 7

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Referred By	DR. M DAS	Sample Rec.Date	19/Jan/2025 05:28 PM
Client Code/Name	AP041308 LIFE CARE PATHOLOGY CENTRE	Report Date	19/Jan/2025 07:01PM
Ref. Lab/Hosp			
Panel Address	12/2 MAHENDRA MUKHARJEE SARANU , UTTARPARA WEST BENGAL		

Test Name With Methodology	Result	Unit	Biological Ref.Interval
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.IMMUNO BIOCHEMISTRY-1

Creatinine

Serum Creatinine.	0.63	mg/dL	0.5-0.9
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Serum, Jaffco

Comments:

The assay of creatinine in serum is the most commonly used test to assess renal function. When creatinine levels rise to abnormal levels, it may be an indication that the kidneys are not properly filtering the blood. Elevated creatinine can also be tied to dehydration, diseases that cause muscle problems, and some complications during pregnancy. If creatinine levels are lower than expected, it may be a sign of malnutrition or conditions that provoke the loss of muscle mass.

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Lab No 26722501190003
Reg Date 19/Jan/2025 11:30AM
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Report Date 19/Jan/2025 07:19PM

Test Name With Methodology	Result	Unit	Biological Ref.Interval
CRP (C Reactive Protein) Quantitative			
CRP Particle enhanced immunoturbidimetric assay.	0.0798	mg/dL	< 0.5

Comment:

A C-reactive protein (CRP), a type of acute phase protein, is used to identify inflammation or infection in the body. CRP is an early indicator of these problems and its levels can rise quickly.

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Report Date 19/Jan/2025 07:21PM

Test Name With Methodology	Result	Unit	Biological Ref.Interval
Glucose Fasting (Blood Glucose Fasting)			
Blood Sugar Fasting Plasma Fluoride, Hexokinase	92	mg/dl	70-100

COMMENTS:

Fasting Blood Sugar/Glucose test. A blood sample will be taken after an overnight fast. A fasting blood sugar level less than 100 mg/dL is normal. A fasting blood sugar level from 100 to 125 mg/dL is considered prediabetes. If it's 126 mg/dL or higher on two separate tests, you have diabetes. (**American Diabetes Association**)

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Ref. Lab/Hosp		Report Date	19/Jan/2025 08:10PM
Panel Address	12/2 MAHENDRA MUKHARJEE SARANU , UTTARPARA WEST BENGAL		

Test Name With Methodology	Result	Unit	Biological Ref.Interval
Rheumatoid Factor (Quantitative)			
Rheumatoid Factor (Quantitative) Serum, Immunoturbidimetric	231	U/mL	<14

COMMENT:

Rheumatoid factor (RA Factor) is the autoantibody (antibody directed against an organism's own tissues) that was first found in rheumatoid arthritis (RA). It is defined as an antibody against the Fc portion of IgG (an antibody against an antibody). RA factor and IgG join to form immune complexes that contribute to the disease process. Rheumatoid factor is an antibody that is detectable in the blood of approximately 80% of adults with rheumatoid arthritis. Sometimes rheumatoid factor can be detected in the blood of normal individuals and of those with other autoimmune diseases that are not rheumatoid arthritis. In people with rheumatoid arthritis, high levels of rheumatoid factor can indicate a tendency toward more aggressive disease and/or a tendency to develop rheumatoid nodules and/or rheumatoid lung disease

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